CVSHealth

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June 16, 2025

Robert F. Kennedy, Jr.
Secretary, Department of Health and Human Services

Dr. Mehmet Oz
Administrator, Centers for Medicare & Medicaid Services

Stephanie Carlton
Deputy Administrator, Centers for Medicare & Medicaid Services

Steven Posnack, MS, MHS
Principal Deputy Assistant Secretary for Technology Policy, Principal Deputy National Coordinator for Health Information Technology

Centers for Medicare & Medicaid Services (CMS)
Department of Health and Human Services (HHS)
Attention: CMS–0042–NC, Mail Stop C4–26–05
7500 Security Boulevard
Baltimore, MD 21244–1850

Re: Request for Information; Health Technology Ecosystem (CMS–0042–NC)

Dear Secretary Kennedy, Administrator Dr. Oz, Deputy Administrator Carlton, and Principal Deputy Assistant Secretary Posnack:

Thank you for the opportunity to comment on the Centers for Medicare & Medicaid Services (CMS) and the Assistant Secretary for Technology Policy (ASTP)/Office of the National Coordinator for Health Information Technology (ONC) (collectively, ASTP/ONC), Health Technology Ecosystem Request for Information (RFI) seeking information on effective and responsible adoption of technology can empower patients to make better decisions for their health and well-being.[1]

CVS Health serves millions of people through our local presence, digital channels, and our nearly 300,000 dedicated colleagues, which includes more than 40,000 physicians, pharmacists, nurses, and nurse practitioners. Our unique healthcare model gives us an unparalleled insight into how healthcare systems may be better designed to eliminate barriers to care access and how implementation of innovative services may better serve individuals' personal health needs through being a trusted partner for every meaningful moment of health.

[1] 90 FR 21034, May 16, 2025.

We appreciate the CMS and ASTP/ONC request for feedback on ways to best advance a seamless, secure, and patient-centered digital health infrastructure. Many agree the current system can be disconnected and cumbersome to interact with. This request offers a ripe opportunity for all health care stakeholders to create digital solutions with appropriate connection points that support data interoperability, such as using Fast Healthcare Interoperability Resources (FHIR) connecting application programing interfaces (APIs) to share health care information. We believe standards-based APIs, combined with elements of the 21st Century Cures Act[2] can accelerate reshaping the ecosystem to an interactive, real-time data exchange and better health care experiences.

When it comes to digital health, one of the main friction points for consumers is the ability to easily find health information, access tools which support consumer engagement, and the ease in which the consumer can connect. We believe real-time connectivity and integration across all healthcare stakeholders is essential for a simplified and individualized consumer experience.

CVS Health is supportive of this transformational shift. We are making significant investments in technology to deliver a reimagined consumer directed experience – focusing on meeting the consumer where they are, when they want to engage, and how they want to interact with us. [3] As digital interactions become a preferred channel for our consumers, our current and future investments will create a fully integrated experience across a consumer's entire health journey, consisting of a seamless experience across our digital (mobile applications, websites, artificial intelligence (AI) assistants), call centers and physical channels, such as community stores, clinics, and provider offices.

We believe modern digital technologies will empower consumer-directed connections, making health care information accessible not just for the consumer, but their providers and payers. Aligned with this vision, CVS Health is ensuring that the reimagined consumer experience platform has a natively open architecture with the ability to integrate with any payer, provider, pharmacy benefit manager, or pharmacy, producing a transportable and interoperable patient health record.

Producing an open architecture environment also accelerates efforts to address many consumer and provider friction points across the health care system, enabling consumers to access their health care information on demand and interact confidently, across the entire health care digital ecosystem, and the CVS Health app is evolving to be the one-stop gateway for digital health point solutions.

CVS Health has created multiple consumer engagement points for managing individual health care information and care benefits which lead to better health care outcomes. The availability of these digital interactions provides insights into key health actions, personalized care navigation, access to real-time health information support, and individual claims management for the consumer. Some actions include:

- Consumer cost tracking, comparison and estimation tools, including assistance with managing a claim, which are accessible to members through the website or mobile apps.

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[2] Pub. L. No. 114-255, available at https://www.congress.gov/114/plaws/publ255/PLAW-114publ255.pdf.

[3] Khemiani, CVS lays out $20B plan to modernize US consumer healthcare experience, Yahoo Finance, June 3, 2025, available at https://finance.yahoo.com/news/exclusive-cvs-lays-out-20b-plan-to-modernize-us-consumer-healthcare-experience-154127371.html.

- Enhanced insights from Aetna for providers that help create a more comprehensive view of a patient encounter, rather than a single interaction.
- Advanced provider search functionality, tailored to a member's unique needs.
- Responsible-AI enhanced member services, care management and provider services, enabling more efficient, effective and personalized care navigation.
- Personalized multi-channel and timely outreach to help consumers make care decisions, that create better health outcomes and save money.

Together, these and our other digital health investments and actions represent a commitment from CVS Health to truly transform health care as an industry leader, disruptor, and demonstrates our willingness and desire to partner with other stakeholders, including federal and state administrations, to make that happen. We are encouraged by the direction and goals CMS and ASTP/ONC have outlined, as well as the recently announced CMS commitment to move forward on a number of initiatives designed to help build the foundational infrastructure needed for a healthcare system.[4]

To achieve these goals, a clear regulatory environment and a sound foundation is necessary to drive large-scale a consumer-centric system, including cross-industry partnership, to move all stakeholders in a transformative direction. Therefore, we believe the following elements are essential to develop a system which can enhance the health care navigation experience, increase access and exchange of health care information, and realize improved health care outcomes and cost reduction.

- **Implementing incentives which encourage all stakeholders to switch to modern data exchange technology, including the development and adoption of real-time FHIR based APIs.**
- **Supporting public/private partnerships which bolster interoperable mechanisms for data access and sharing between patients, caregivers, providers, and payers.**
- **Fostering utilization of digital technologies which enhance workflows, focus on patient care improvement, promote population health care management, and reduce burdensome steps within value-based care models.**
- **Maintaining a national privacy framework and existing health care data privacy protections under the Health Insurance Portability and Accountability Act (HIPAA) Privacy and Security Rules.[5]**

Finally, CVS Health is aligned with CMS' and ASTP/ONC's vision and appreciate the willingness to engage and partner with stakeholders in advancing the use health technology innovations to enhance patient care and promote efficiencies. We urge CMS to continue engaging with the private sector stakeholders to build a solid foundation, fostering trust, and facilitating good public policies where digital technologies can succeed.

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[4] CMS, CMS Building Foundational Infrastructure for Digital Healthcare Ecosystem, June 3, 2025, available at https://www.cms.gov/newsroom/press-releases/cms-building-foundational-infrastructure-digital-healthcare-ecosystem.

[5] Codified at 45 CFR Parts 160, 162, and 164.

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Included is an appendix with our specific comments and recommendations for consideration. Please do not hesitate to contact us with any questions about these comments.

Sincerely,

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CVS Health

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Request for Information: Health Technology Ecosystem
Appendix

Please note: A single response may be replying to multiple RFI questions as topic areas and approaches may overlap.

I. Patients and Caregivers

PC–1. What health management or care navigation apps would help you understand and manage your (or your loved ones) health needs, as well as the actions you should take?

PC–2. Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)? If so, what are some examples of benefits it has provided?

PC–6. What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

PC–12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

CVS Health's unique healthcare model provides insights into how healthcare systems may be better designed to eliminate barriers to care access and how to innovate in ways that better serve individuals' personal health needs. Through day-to-day engagement with millions of consumers, CVS Health gathers insights on consumer needs, and solves for friction points (i.e., unpredictable costs, long appointment wait times, lack of care coordination and care management options). We listen to feedback and strive to develop an open and interoperable ecosystem where the consumer owns their experience.

Most recently, in January 2025 CVS Health launched the CVS Health app, building on a previous app, now redesigned to become a single app for consumers' whole health. The CVS Health app is a consumer-focused app that will have the ability to be the center of all medical and pharmacy records, even for those consumers that are not CVS Health customers or members. Features are not just for patients, but for caregivers as well, and include:

- Ability to manage prescriptions and immunization scheduling for the whole family, tracking prescription order status and cost.
- Scheduling options for in-person and virtual MinuteClinic appointments and to review lab results and visit histories.
- Access to health spending and benefit details for eligible CVS Caremark and Aetna members.
- Options for timely, personalized updates, including a personalized home screen featuring a health "to do" list and recommendations, including reminders to refill medications, and updates on upcoming appointments.

Over the past year, CVS Health has seen a greater interest in the CVS Health app, which currently has over 14 million monthly users. Continuing this momentum, new CVS Health app capabilities will evolve to include:

- Managing chronic health conditions will become easier, with access to personalized recommendations and educational content that guide patients along their care journey and provide details on relevant cost savings and copay assistance programs.
- Conversational AI chat will enable patients to check medication refills, the status of their orders

and more.

- Continued and enhanced integration with Aetna, Caremark and other parts of CVS Health will create a unified experience for consumers, when appropriate.

CVS Health intends to build a truly integrated ecosystem, open to all, based on a set of common standard interoperability functions and agreed-to health information privacy and security protections. CMS and ASTP/ONC can support this effort by driving alignment: clear policies that encourage all stakeholders to embrace data interoperability and the necessary interoperable technology, facilitate cross-sector efficient care navigation, and enable scalable, high-volume data exchange.

II. Providers

PR–2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

State level barriers impede smart and efficient technology investments, improvements and modifications that would enhance the patient and provider experience. For instance, some states allow or prohibit certain functions and capabilities depending on the state regulations or the specific member benefit type, i.e., incorporation of certain responsible AI technologies, such as informational chat bots or AI Assistants.

Another example is a prohibition on the use of prior-authorization approval technologies for Medicaid members in certain states, even though the incorporation of these technologies seeks to reduce administrative burden and improve time from the prior authorization request to approval. These constraints impact what digital capabilities can be used, impede the flow of relevant health care information, and limit the ability to improve the member and provider experience.

There is an opportunity for CMS and ASTP/ONC to signal support and adoption of technology inclusion across populations and different benefit constructs, especially when the use improves the ability to share information and reduce system inefficiencies.

PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem?

Aligned with seeking a more consumer-centric approach, Aetna is also committed to improving the member and provider experience with technology and operational advancements aimed at enhancing quality of care, addressing areas of frustration and inefficiency experienced by providers and members.

Aetna has invested in electronic health records (EHRs) to enable seamless communication and data exchange with any other provider utilizing the same EHR system. An example of how this works is within CVS Health Virtual Primary Care. Virtual Primary Care consists of a connected care team, including nurses and pharmacists, who connect via an interoperable electronic health record that allows members to transition between virtual and in-person care, with clinical data shared with other providers (virtual and in-person) as directed by the consumer. By combining health services, a coordinated Care Team has a comprehensive view of clinical data and care activities, enables providers to deliver a more personalized experience and improving outcomes.

Another example is recent advancements in Aetna's prior authorization care bundles. A prior authorization bundle involves obtaining a single approval for multiple related services expected during a specific patient episode (such as cancer care), streamlining the process, and reducing administrative burdens. This approach can speed access to care for members by quickly approving coverage requests from providers, to help them provide the best evidence-based care. The goal is to ensure high-quality

care by focusing on the needs of patients, reducing friction and confusion in the healthcare process, and helping to make the provider administration process more efficient.

These health care advancements help to foster better care coordination and communication across different providers and care settings, including specialized medical care, and demonstrate how we are innovating to support providers and serve our members at every stage of their health. As CMS and ASTP/ONC consider next steps in supporting an enhanced the health care navigation experience for providers, we urge CMS and ASTP/ONC to leverage established technologies and frameworks to ensure smoother integration and build on accepted user platforms to promote more rapid adoption of digital health interactions.

III. Payers

PA-1. What policy or technical limitations do you see in TEFCA? What changes would you suggest to address those limitations? To what degree do you expect these limitations to hinder your participation in TEFCA?

CVS Health supports the goal of establishing a nationwide "network of networks" to support data sharing. To address the current limitations of Trusted Exchange Framework and Common Agreement (TEFCA), we suggest improving the cumbersome policies and technical standards to facilitate payer participation, currently, the cost and fees associated with data exchange is an issue is a barrier to adoption.

PA-2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers and other payers, similar to the Blue Button 2.0 and Data at the Point of Care ACPs released by CMS?

Facilitate Public-Private Partnerships

As we look at the health tech ecosystem today, we see tremendous progress – and much more work to be done. Moving toward the future together, it is critical that CMS work in partnership with the private sector to reach common goals. CVS Health is committed to doing its part to improve the patient care journey experience and create a health care system that works for patients, rather than requiring patients to do all the work.[6] This will require shifting from a system that is merely responsive to patient needs to one that is proactive in engaging with patients throughout the health care experience. It will also require a collective reimagining of the current patient experience to ensure that patients can engage with the health care ecosystem using technologies that they already know and love.

Avoid Reinventing Systems Consumers Know and Use

CVS Health strongly believes that CMS must avoid reinventing current systems that consumers use to manage their health and health care. Reinventing or unnecessarily duplicating systems leads to increased complexity, reduced interoperability, and heightened user confusion, ultimately undermining consumer confidence and adoption rates. Leveraging established technologies and frameworks ensures smoother integration, enhances user acceptance, and promotes more rapid, cost-effective innovation, thus accelerating meaningful improvements in health outcomes and patient satisfaction.

[6] Khemiani, CVS lays out $20B plan to modernize US consumer healthcare experience.

Support the Use of FHIR Across the Healthcare System

One of the most impactful things that CMS could do to encourage payers to accelerate the implementation and utilization of APIs for patients, providers and other payers is support FHIR-based health information exchange. This will require robust technology infrastructure, including the necessary data elements readily available EHR without extra fees, a national network to support data exchange, and a directory of digital endpoints.

PR. 9: How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

PA-3. How can CMS encourage payers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins?

CVS Health supports the development of a digital consumer identity which aids in consumer ownership of their information and sharing such information at the consumers direction. As CMS moves forward with the commitments announced on June 3, 2025, we encourage CMS to consider appropriate consumer protections and information security. One way to modernize HIPAA security practices would be to consider confirming digital consumer identity technologies, like those referenced, as HIPAA-compliant.

Digital identity credentials must be shown to meet HIPAA, as well as state law, requirements for how PHI is accessed, stored, and shared so that consumers have confidence in the security of their personal health information. CMS could also work with the National Institute of Standards and Technology (NIST) to update its digital identity guidelines and develop a framework for voluntary adoption in the health care sector. We also encourage CMS and ASTP/ONC to think about the challenges of digital identity broadly when exploring solutions to facilitate API adoption.

With respect to payer-to-payer (P2P) consent management, a safe harbor for payers permitting members to enroll in P2P API data exchange when they log in to their online member account would allow payers to use proven processes and protected technologies to collect the necessary information from patients.

Another important function to consider is the acceptance of digital identification (ID) cards by providers, including Medicare. Providers can not immediately see benefit allowances, prior authorization indicators, or the status of claims processing when a member uses a paper ID. Using a digital ID card, a health plan can share certain benefit details which may be helpful when a patient and provider are discussing care options. By accepting digital ID cards, CMS would signal an important shift from paper to digital ID cards, allowing information to be accessed at the right time, and possibly even accruing administrative savings through digital efficiencies.

PA-4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

VB–15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources?

Every patient and consumer should be able to easily find a clinician or facility skilled in the type of care they seek. We urge CMS to work in partnership with the private sector to establish a national directory of plans and providers that solves not only the need for digital endpoints but also demographic data.

To aid with alleviating provider friction points and improve the accuracy of provider credentialing and timely provide communications, CMS should develop the nationwide provider directory using a

federated model supported by a public-private partnership rather than centralized control by CMS. We believe that such an approach is necessary to streamline collection of this information and improve its accuracy and allows connections with other data management systems/EHRs, and potentially scheduling/practice management systems. Certification Program criteria should require Certified Electronic Health Record Technology (CEHRT) developers to build necessary connection to a national provider directory and CMS could build on existing work to develop FHIR-based standards for healthcare directories. This includes Validated Healthcare Directory (VHDir) Implementation Guide (IG); API for local directories to make verified data available to stakeholders: the National Directory Endpoint Query IG, the National Directory Exchange IG, and the National Directory Attestation and Validation IG.

The success of a national provider directory hinges on the establishment of trusted governance. Without a central, transparent authority to define standards, oversee data accuracy, and ensure accountability, provider information can quickly become outdated, fragmented, or inconsistent across systems. Trusted governance is essential to set clear expectations for data contributors, maintain data integrity, and align incentives across payers, providers, and technology vendors.

IV. Technology Vendors, Data Providers, and Networks

TD-4. How can CMS better encourage use of open, standards-based publicly available APIs over proprietary APIs?

Use Existing Standards Development Processes and Released Standards

As a starting point, CMS should utilize existing standard development processes, embedding released standards-based guides in their requirements where possible. Establishing a baseline framework of elements and/or standard information for all stakeholders to use and ingest across systems is a critical starting point. We recognize that some functionality may need to be phased in for those who are not able to adopt as quickly, but CMS should establish firm deadlines and require all to meet those deadlines as expeditiously as possible while allowing those that are able to adopt standards to move ahead.

Establish Clear, but Achievable, Deadlines for Clean/Usable Data

Establishing clear deadlines for clean and usable data in quality reporting is essential to ensure accountability, consistency, and timely action across all stakeholders in the health system. Without firm timelines, data collection and validation processes may be delayed or deprioritized, leading to incomplete or inaccurate submissions that undermine the reliability of quality measures. Deadlines create structured expectations that drive coordination among clinical, information technology, and administrative teams, promote adherence to data standards, and support the continuous improvement of health outcomes. Moreover, timely availability of high-quality data is critical for informing decision-making at the provider, payer, and policy levels, enabling more responsive care delivery and effective oversight.

Support Bulk FHIR

Bulk FHIR enables scalable, high-volume data exchange, which is essential for advancing population health management, quality reporting, and value-based care. Bulk FHIR enables the efficient transfer of large datasets—such as those needed for clinical quality measures, health equity analysis, and program oversight—across disparate health systems and payers. Without robust support for this standard, data exchange remains fragmented and burdensome, limiting the ability of providers, states, and federal agencies to generate actionable insights. CMS' leadership in promoting Bulk FHIR adoption by EHRs, payers and national exchange networks would accelerate interoperability, reduce administrative overhead, and ensure more timely, accurate data to inform health policy and improve outcomes at scale.

V. Value-Based Care Organizations

PA-5. What are ways payers can help with simplifying clinical quality data responsibilities of providers? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

PC-8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

We encourage CMS to work with payers and providers on data systems to support the timely attribution of beneficiaries to health plans and Medicare Advantage value-based care arrangement patient panels. For example, Oak Street Health focuses on delivering high-quality primary care to over 350,000 patients, many of whom are enrolled in a Medicare Advantage plan. When an existing Oak Street Health patient changes plans during open enrollment, that member or the individual assisting the member with open enrollment may not attribute their Oak Street Health provider as their primary care provider. As a result, it may take several months until Oak Street Health is re-attributed as the patient's provider under the new plan, yet the member remains an Oak Street health patient.

Potential solutions may include developing a single source where both payers and providers can verify patient attribution, i.e., a public-private clearinghouse for plan enrollment verification and/or the use of FHIR APIs to support the bulk data exchange of updated member enrollment and attribution status. Access to accurate patient attribution data is important to value-based care providers and is key to identifying appropriate clinical actions to take, especially in clinical cases where timely interventions may be needed, such as following a hospital discharge, long-term acute care (LTAC) admissions and approvals, and within multidisciplinary care teams.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

CMS and other payers have an important role in encouraging all stakeholders to participate in adopting technology to support data interoperability. This includes ensuring that there are appropriate incentives and workflow accountability for collecting, storing and sharing key health information, as well as minimizing vendor fees where possible, recognizing the complexities of applicable state law, and aligning quality measurement across markets.

<u>Ensuring Appropriate Incentives</u>

Ensuring appropriate financial incentives is critical to promoting accurate and meaningful quality measurement in health care. When incentives are misaligned, providers may not prioritize investments in the systems, training, and workflows necessary to collect and report reliable data. Conversely, well-structured incentives can drive improvements in data accuracy, foster a culture of continuous quality improvement, and reduce the administrative burden of reporting. Financial models should reward not just performance outcomes, but also data completeness, timeliness, and integrity—recognizing the effort required to build robust digital infrastructure. Aligning incentives with these goals supports more equitable, actionable, and patient-centered quality measurement across the health care system.

<u>Create Workflow Accountability</u>

Workflow accountability is essential for accurate and timely clinical quality measure (CQM) reporting. Clear delineation of roles and responsibilities across clinical, administrative, and information technology teams ensures that data is captured, validated, and reported consistently and in alignment with

measure specifications. Without structured workflows and accountability mechanisms, gaps in documentation, delays in data submission, and errors in measure calculation can compromise data integrity and reduce the effectiveness of quality improvement efforts. Embedding CQM tasks into routine clinical operations, supported by training and oversight, helps foster a culture of shared responsibility and continuous performance improvement.

Minimizing Vendor Fees

ASTP/ONC should work with its Recognized Coordinating Entity (RCE) to ensure stakeholder can access data for quality measurement without undue fees. Vendor fees significantly reduce health plans' ability to participate in TEFCA and share clinical quality data.

CMS should also explore ways to use the APIs created by its Interoperability rules to support the exchange of digital quality measures, including FHIR-based electronic clinical quality measures (eCQMs) such as allowing plans to use an API to access EHR data needed for Healthcare Effectiveness Data and Information Set (HEDIS) measures.

Recognizing State Challenges

State laws can complicate the digitization of CQMs due to varying data sharing restrictions, consent requirements, and privacy protections that differ across jurisdictions. In some states, stringent regulations limit the ability to exchange health information, particularly behavioral health or sensitive patient data, even when federal standards like HIPAA would permit it. These legal constraints hinder the development of unified, interoperable data systems needed for end-to-end digital quality reporting. Additionally, conflicting definitions of authorized use, patient consent, and provider obligations create uncertainty for health systems and health information exchanges (HIEs), discouraging investment in digital infrastructure. As a result, states often face fragmented data environments that impede their ability to implement standardized, scalable digital CQM solutions.

Aligning Quality Measurement Across Markets and Payers

CVS Health supports quality measurement alignment. One of the most significant issues raised by providers is the creation of various individual payer mechanisms and approaches to capture the quality measures health plans are accountable for. For example, Medicare Advantage star measures, Medicaid quality measures, and commercial HEDIS measures often don't line up. Therefore, we try to align our value-based care (VBC) contracts with the measures we are held accountable for, but given disparate measures exist across these programs, quality measures may present differently for providers who see patients across these lines of business.

While CMS and health plans continue to work toward measure alignment, there are still many states engaged in their own quality measure efforts, leading to misaligned measures and measure specifications across state-level programs and CMS initiatives. This misalignment impacts payers and providers, who must try to reconcile these measures in value-based care arrangements.

We recommend CMS first prioritize measure alignment with states and across CMS programs. This would provide health plans and providers with a clearer pathway toward true measure alignment.

TD-8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

Support Trusted Exchanges

CEHRT can be strategically narrowed to focus on trusted health information exchange by prioritizing certification criteria that directly support interoperability, data integrity, and security. This targeted approach would streamline program requirements to emphasize capabilities such as standardized APIs (e.g., FHIR), support for bulk data exchange, patient access provisions, and adherence to recognized data formats and vocabulary standards. Narrowing the scope would reduce unnecessary administrative burden on developers while reinforcing the program's core purpose—ensuring that health IT systems enable seamless, secure, and reliable information exchange. By concentrating on functionalities that facilitate trusted data sharing, the program can better align with federal interoperability goals, foster innovation, and accelerate nationwide progress toward a more connected and transparent healthcare system.

Ensure Data Protection and Security

Health data protection and security should remain central to any data interoperability transactions across the health care ecosystem and those that interact with personal health information (PHI). We encourage CMS and ASTP/ONC to work with relevant federal agencies and stakeholders to identify proposals to modernize HIPAA, expand HIPAA application to covered entities which interact with PHI, and provide clear guidelines and guardrails for interacting with PHI under new technology constructs and new care delivery models. These actions will encourage trust between stakeholders, including individuals sharing access to their health information with existing and new technology platforms that emerge as a result of the goals and efforts CMS and ASTP/ONC adopt.

VB–3. What are essential health IT capabilities for value-based care arrangements?

One important health IT capability for value-based care arrangements is the ability to support electronic consultations (eConsults) for patients who may need to seek consult or care from a specialist provider. Oak Street Health introduced enhanced eConsults & Referral Management to their primary care providers (PCPs) seeking to increase patient access to specialized care, improved health care outcomes, and realize administrative efficiencies.[7]

Oak Street Health receives and processes a volume of specialist referrals on behalf of patients daily and realized there were two issues with the referral process that needed to be addressed. First, Oak Street Health noticed over-referring for cases that could be managed in-house through primary care. Secondly, Oak Street Health members may have unique mobility and transportation challenges which can impact their ability to access external referrals. Implementing an eConsult technology provided a tool for both patients and providers to minimize these challenges.

The eConsult technology offers secure, text-based communications facilitating direct dialogue between PCPs and specialists where PCPs can open an eConsult seamlessly in the EMR and submit comprehensive patient information, including history of present illness (HPI) and specific clinical questions to a specialist. In turn, the specialist provides timely, guideline-based care plan recommendations, empowering PCPs to manage complex conditions within the primary care setting.

The implementation of eConsult technology has yielded demonstrable improvements in the referral process, patient care and access, as well as provider satisfaction. For instance, when an eConsult was

[7] Oak St. Health White Paper, The Gold Standard of Advanced Primary Care for Medicare Beneficiaries, May 2025, available at www.cvshealth.com/content/dam/enterprise/cvs-enterprise/pdfs/2025/Oak-Street-White-Paper-2025-v2.pdf

submitted prior to an external specialist referral, the ability to communicate between providers and view patient information prevented the need for that external referral almost half of the time.

This eConsult capability is crucial within a value-based care framework, as it directly supports improved health outcomes, reduces unnecessary specialist visits, and lowers overall care costs by preventing acute escalations and ensuring appropriate care at the most effective site. It aligns with the core tenets of value-based care by prioritizing patient access, care coordination, and cost-efficiency.

As CMS continues to advance the utilization of digital health technologies, we urge CMS to explore incentive programs that promote widespread adoption of eConsult models, particularly within primary care organizations committed to value-based care, recognizing their potential to enhance access and system-wide efficiency. Additionally, policies should encourage the seamless integration of eConsult platforms within EHR systems to achieve optimized workflows and reduce administrative burden.